

— The Station —

Engaged with the evidence.

February 2026's Edition

TEAM HUDDLE: EARLY PREGNANCY

Early pregnancy care resides at the intersection of rapid diagnosis, complex counselling, and long-term psychological impact. While a critical window of development, ectopic pregnancy remains a leading cause of maternal morbidity—occurring in 1% to 2% of pregnancies and accounting for nearly 6% of pregnancy-related deaths. As global caesarean rates rise, the increasing incidence of caesarean scar pregnancies (occurring in roughly 1 in 2,000 gestations) requires us to re-evaluate their role within the placenta accreta spectrum. This month, we bridge clinical learning with compassionate practice, examining diagnostic pitfalls, CTG monitoring in high-risk multiples, and the vital management of psychological sequelae following loss.

The "Disappearing" Membrane

Presented by **Zoe Slack**

BACKGROUND

31-week MCDA scan: intertwin membrane difficult to see and free-floating, raising concern for septostomy.

INTERPRETATION

Septostomy effectively converts MCDA twins into a single-sac pregnancy with much higher cord-entanglement risk; any "disappearing" membrane should trigger senior review and closer surveillance.

OUTCOME

At 34 weeks, SROM was followed by membrane entanglement around one twin's leg causing transient ischaemia; both babies were delivered safely by caesarean.

💡 CLINICAL PEARL

A disappearing membrane is a high-risk red flag. This image (Fig. 2) shows the physical cause of the foetal distress: the ruptured membrane allowed one cord to wrap around the other four times. This entanglement, tightened by membrane debris, caused both cords to be compressed simultaneously during uterine activity, preventing the twins from tolerating contractions.

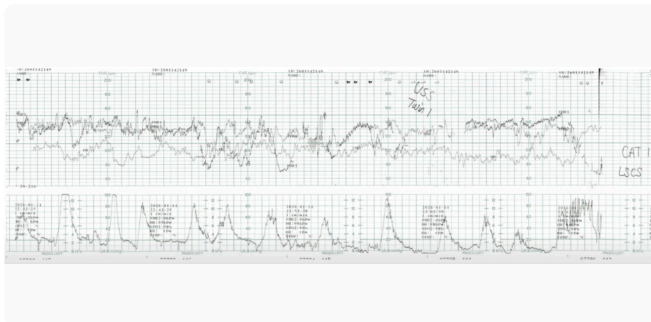


Figure 1. CTG trace showing acute foetal distress during labour.

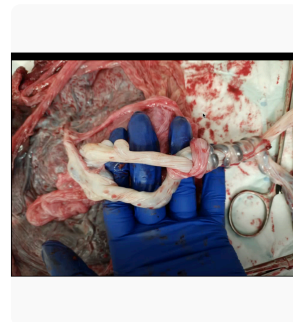


Figure 2. Placental cord and membrane entanglement.

Posttraumatic stress, anxiety and depression following miscarriage and ectopic pregnancy: a multicentre, prospective cohort study

Am J Obstet Gynecol, 2019 · Presented by Shehani Alwis

KEY FINDINGS

Large multicentre cohort of women after miscarriage, ectopic pregnancy, molar pregnancy and PUL followed at 1, 3 and 9 months with validated PTSD, anxiety and depression scales.

KEY RESULTS

High rates of PTSD, anxiety and depression shortly after early pregnancy loss, with a substantial minority still symptomatic at 9 months; many women declined participation because they did not want to be reminded, highlighting classic PTSD avoidance.

TAKE HOME

Do not assume emotional recovery tracks physical recovery: normalise talking about PTSD, anxiety and depression after early loss and build signposting to psychological support into routine follow-up.

[Link to paper ↗](#)

NICE Ectopic pregnancy and miscarriage: diagnosis and initial management

Presented by Ashlesha Sardesai

Imaging:

TVS is first line.
Laparoscopy is not the diagnostic gold standard.

Sac appearance:

True IUP: double echogenic ring.
Pseudosac: single ring and common in ectopic pregnancy.

Tools:

Use the sliding sign to confirm a tubal mass.
Use serial beta-hCG only for PUL.
Do not use serum progesterone to locate the pregnancy.

Bottom line:

If you cannot clearly see a double ring, do not label it an IUP.
Manage as PUL until the location is proved.

[Link to guideline ↗](#)

Caesarean Scar Pregnancy Spectrum: Standardised Classification and Terminology

J Ultrasound Med, 2026 • Presented by Lucy Thorn

KEY FINDINGS

CSP is increasingly seen with rising caesarean rates and sits on the same spectrum as placenta accreta spectrum (PAS). This paper proposes a three-type system based on where the sac sits relative to the scar and cavity.

KEY RESULTS

Type 1 – on the scar, mostly in the cavity: best chance of reaching term.

Type 2 – in the niche: high risk of PAS.

Type 3 – exogenous, bulging towards the bladder: highest risk of rupture and major haemorrhage.

TAKE HOME

Classifying CSP as type 1–3 at first-trimester scan helps stratify risk, support shared decision-making and plan follow-up in centres experienced with PAS before catastrophic bleeding occurs.

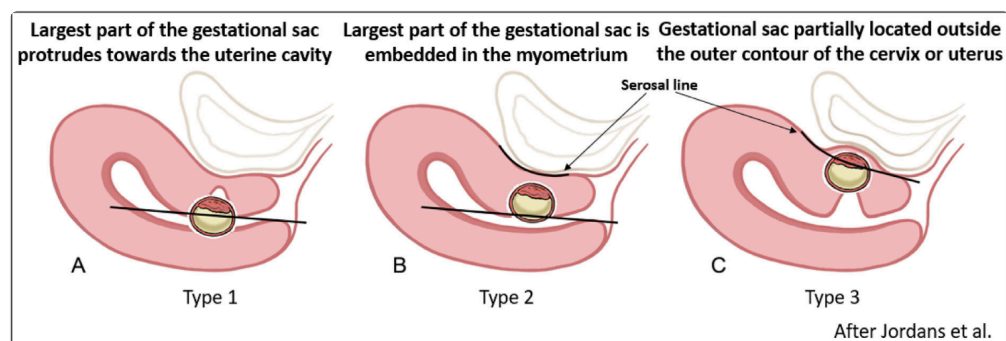


Figure 3. Definition and sonographic reporting system for caesarean scar pregnancy in early gestation: the modified Delphi method

[Link to paper ↗](#)

The "Frog Test" of the 1940s

1940s

Before the era of simple plastic dipsticks, clinicians relied on the Hogben test. To confirm a pregnancy, a woman's urine was injected into an African clawed frog; if the pregnancy hormone hCG was present, the frog would ovulate and lay eggs within 24 hours. When modern chemical tests arrived in the 1960s, these "living lab tests" were no longer needed. Thousands were released into the wild, inadvertently spreading a deadly fungus (chytrid) that has since caused devastating population declines and extinctions of amphibians worldwide. It is a sobering reminder that our instant diagnostic tools are built on decades of animal-based science—and a global ecological footprint.



Figure 4: African clawed frogs used in the Hogben pregnancy test.

[View on PubMed ↗](#)

MARCH 2026 SCHEDULE

Upcoming Meetings

March's Theme: Menopause

DATE	TIME	EVENT	TOPIC	LOCATION
Mar 3 Tuesday	08:30–09:00	CTG	By Snehal Kolluru	Microsoft Teams
Mar 10 Tuesday	08:30–09:00	Gynae JC	Fezolinetant for treatment of symptoms associated with menopause (Romana Cuffolo)	Microsoft Teams
Mar 17 Tuesday	08:30–09:00	Guidelines Session	NICE Menopause & RCOG HRT (Niluka Kanaganayagam)	Microsoft Teams
Mar 24 Tuesday	08:30–09:00	Obs JC	TBD (Vishnu Shivanand)	Microsoft Teams

REMINDER

MDT Teaching

Tuesday morning 08:30–09:00

[Join MDT Teaching ↗](#)



